

Eating Disorders & Pregnancy

For someone who has an eating disorder, or who has recently recovered from an eating disorder, becoming pregnant can be a particularly vulnerable time. This is because pregnancy is associated with many changes related to food, eating, body weight and shape. For example:

- Experiencing changes in body shape and increasing weight
- Cravings and physical drives to eat more
- Morning sickness which may involve vomiting or food aversions similar to disordered eating behaviours
- Other people (even strangers!) thinking that it's ok to comment on or touch a pregnant woman's body
- Having to limit exercise - pregnant women are recommended to do only a moderate amount of aerobic exercise, and some activities are not advised



Potential Risks

Having an eating disorder during pregnancy can pose increased risks to the health of both mother and baby. Mother and baby may be at increased risk if:

- Mother is underweight or does not gain enough weight during pregnancy
- Mother's nutrition is not sufficient during pregnancy and therefore baby does not receive adequate nutrition
- Mother is engaging in binge eating or purging behaviours

Specific risks relevant to your situation should be discussed further with your healthcare provider.

Changes in Nutritional Requirements

A mother's body needs extra energy and nutrients during the pregnancy and lactation period in order to maintain her wellbeing as well as support the baby's healthy growth.

- In the first trimester, folate, vitamins A and B6, and iron are particularly important for the development of the heart, brain and bones, as well as establishing the placenta and blood supply
- In the second and third trimesters, extra energy and nutrients including protein, carbohydrates, and vitamins C and D are required to support development of the foetus' brain, nervous system, muscles and tissues, fat storage and facial features
- Breastfeeding mothers will continue to need



additional energy in the form of food throughout nursing

Weight Gain in Pregnancy

Weight gain (including fat storage) is a natural and essential biological process during pregnancy. Some people can find the changes in the number on the scale distressing, so it is important to remember that weight gain is appropriate and necessary.

Factors that contribute to weight gain include:

Your baby (~3-3.6kg)
The placenta and amniotic fluid (~1.5kg)
Uterine muscles (~1kg)
Increased blood volume (~1.4-1.8kg)
Fluid retention (~1-1.5kg)
Breast tissue (~0.5-1kg)
Fat storage for breastfeeding (~2.7-3.6kg)

At times the number on the scale can increase rapidly. This is due to normal growth fluctuations and hormonal changes. It's especially important to maintain enough water intake to avoid dehydration and rebound swelling as the body seeks to secure nutritional resources.

Accessing Support

It is important that women who are pregnant (or hope to become pregnant) access support so they can prepare for any challenges they may face. Being open and honest about the support you need is essential and can ensure the safe monitoring of you and your baby.



Support is also available for women who experience an increase in eating disorder thoughts and behaviours after their baby is born. Postpartum can be a particularly challenging time for women, as people often hold unrealistic expectations about returning to a particular body shape or weight.

Options for support can include:

- Attending your GP regularly
- Attending antenatal and postnatal appointments
- Seeing a healthcare professional (e.g., dietician, psychologist) with experience in eating disorders

Importantly, many women with eating disorders cope well with pregnancy, and even reduce or cease their eating disordered behaviours altogether.